

DENTAL ASSISTING NOTES

August 2025

Summary

Dental assisting notes based on work at Children's Dental Associates Inc., town office.
The very basics of setting up, cleaning, sterilization, radiographs, prophylaxis, restoratives, and navigating Dentrax.

Tiffany Taketa

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Rules & Code of Conduct

Clocking in, Timesheets

Glossary

Prophy angle

Cavitron

Slow-speed handpiece

High-speed handpiece

Air/water syringe

Low volume evacuator (saliva ejector, “low-speed” suction)

High volume evacuator (“high-speed” suction)

(pictures or labelled picture of bay?)

Burs

(picture)

Restorative instruments

(picture)

Map

Morning & End of Day Duties

Morning

- **Clock in** – can be 5 min prior or on time
- **Start laundry**
 - Detergent is under the sink, make sure door is closed before starting
 - Caps stay on dryer, towels are kept with the instruments in the bottom drawer, hang jackets in closet
- **Check if gas & water need to be refilled**
 - Water should be up to first line (refill with distilled water and beaker, in cabinet under the trays)
 - Use wrench to turn N2O and O2 on (to the right), tap gauge to move the needle (replace tank if needed), turn off (to the left)
 - To replace gas or O2, unscrew handle on the gauge, remove tank and place in cart next to the cabinet with trays in the hallway, replace with new/unopened tank
- **Fill the BioSonic, set out towels**
 - Close drain on the BioSonic, fill with tap water up to the lower line, pour ~4 oz (2 of the small cups, in cabinet above BioSonic) of general-purpose fluid (blue, cabinet under the BioSonic)
 - Run the unsterilized tools from the day before for 15 min in the BioSonic, if needed
 - Set out a hand towel for tools and instruments to dry, and another for trays to dry
- **Put away sterilized tools**
 - Check that instruments are sterilized: corner pink = unsterilized, gray = sterilized
 - Set aside and take to the counter in the back with trays and napkins:
 - Cavitrons and proph angles – place in each room to put on
 - Cavitron tips, wrenches, (to unscrew cavitron tips) – tips in front, wrenches in back of container on the right side in the back
 - mirrors and explorers (for setting up trays)
 - Put the rest back into the drawers under the counter
- **Prepare trays**
 - Set up dental trays (lay a napkin across the tray, place another napkin on one side and a mirror & explorer pouch on the other), have mirror and explorers on alternate sides when stacking trays to make them more balanced, distribute to bays
 - Take out and prep treatment trays for the morning
- **Prepare treatment rooms**
 - Barriers on HVE, air/water syringes, saliva ejector
 - Barriers on chair buttons (2), overhead light (2), mouse (1)
 - Large barriers on chair and gas tanks
 - Nose for nitrous (keep covered with wrapper and drape over machine)
 - Put proph brushes on proph angles and tips on Cavitrons
- **Turn on computers & TVs**
 - Turn on server, press a key, login, open Appointment Book  & Imaging Bridge 
- **Review appointments for the day** (family files, age, status)
 - Make goody bags for the 1st appointments
- **Restock**

End of day

- Put all handpieces & tools in the sterilizer and start cycle (wrapped pouches > start)
- Raise chairs (press 4)
- Restart computers and turn off servers
- Sweeping → mopping
 - Treatment rooms, sterilization area, break room
 - Get broom and mop from bathroom shower; use a bit of floor cleaning fluid (under the sink)
 - Fill container with water and pour into the mop bucket
 - Can dump water in the toilet after mopping
- Clean the toilet
- Vacuum carpeted areas
- Take out trash
 - Anytime after 5pm = in the corner across from the bathroom by the elevators
 - 1st floor dumpsters in the parking lot if before 5pm

Wednesdays

- Regular morning duties
- Dust
- Wipe counters/surfaces
- Clean out the trap
- With the HVE, suction vacuum cleaner (yellow) then water
 - In the cabinet under the BioSonic

Cleaning, Sterilization

Attire: scrubs, hair tied up

While assisting, always wear PPE (face shield, mask, gloves, jacket)

- Jacket (always have a jacket on to protect your skin from chemicals/blood/saliva/etc.)
- Mask (always wear when seeing patients)
- Face shield or glasses (wear while assisting during cleanings and treatment)
- Gloves – wear gloves while:
 - Assisting
 - Cleaning
 - Don't wear gloves if you are not in the operatory or sterilization area (i.e., when walking out to the waiting room)
 - Change gloves when there is a possibility of cross-contamination (e.g., writing a referral or touching something that was not sterilized during treatment)
 - (need to walk through every task and list when to wear/not wear/change, also depends on the person I think)

Wipe everything that you touch during treatments/cleanings with cavicide

Wipe items that fall on the floor, with cavicide (also replace if disposable) and then change gloves. If you don't need to use it again, leave it on the floor and pick it up later while cleaning.

Cleaning Bays

1. If touched/used, throw away saliva ejector, air/water syringe, HVE tips and barriers, and prophylaxis brush
2. Clean the tray
3. Dispose of all barriers (on the light, chair, mouse, gas tanks, keyboard) that were touched/used/contaminated
4. Wipe any chairs, prophylaxis angles, Cavitron, holders, scavenging circuit (tubes, nose piece, switch – if using nitrous) that were touched and the counters (no need to scrub/repeatedly wipe the same area; larger sideways motions may be faster than small circular ones)
5. Put on new barriers (chair, light, chair control panel, mouse, gas tanks)
 - a. Light – fold the top 1/3rd down on the horizontal edge (place on light with folded edge on top)
 - b. Mouse – fold the top 1/4th down on the vertical edge (place with folded edge near the top of the mouse)
 - c. Keyboard – wrap in cling wrap
 - d. Large barriers for the chair and gas tanks
6. Put in new HVE, saliva ejector, air/water syringe, and Cavitron tips, and prophylaxis brush
 - a. The bevel and hole on the High Volume Evacuator (larger suction) should be facing up
 - b. Push the outer ring on the air/water syringe while inserting the tip and release. Tug the nozzle to make sure it is in place.

- c. Some prophyl angles have a locking mechanism that must be twisted/turned to unlock (look for the circles) and put the prophyl brush on, and lock to make sure it stays in place
7. Make the tray for the next appointment

Cleaning Trays

Prophys:

1. Separate things that can be sterilized (i.e., mirror, explorer, Cavitron tip) from disposable things (i.e., gloves, plastic wrappers, floss, brush, fluoride, paper)
2. Take the tray with mirror, explorer, and Cavitron tip to the sink, scrub/rinse off residue from prophyl paste, blood, etc., put in the basket and start ultrasonic (for 15 min) if the basket is getting full.
3. Clean the tray – Scrub, spray, and wipe trays
 - a. Scrub both sides with dish soap (just a few scrubs are needed)
 - b. Spray both sides at least once with cavicide (for example once in a sweeping motion)
 - c. Leave on the towel against the wall to dry
 - d. Later (once tray is dry), wet a 4x4 gauze with alcohol and wipe down the entire tray(s) with alcohol. Put back into the cabinet to dry.

Treatments:

1. Separate things to sterilize and throw away
 - a. Throw away:
 - i. 2x2s/gauze, cotton rolls, bond brushes and micro-brushes
 - ii. Q-tips, micro/bond brushes
 - iii. Rubber dam
 - iv. Floss (take floss off bite blocks and clamps, using tweezers helps)
 - v. Bands and wedges
 - vi. Bite paper, barriers, napkins
 - vii. Saliva ejector, HVE, air/water syringe tips
 - viii. Riva/Equia/packable/amalgam capsules
 1. Riva/Equia: push button, take out of applicator
 2. Packable: pull off composite gun
 3. All: throw away used capsules
 - b. Biohazard waste bin: lidocaine and needle (in bay or near BioSonic)
 - i. Take the lidocaine out first in case the cover comes off the needle
 - ii. Holding needle away from you, grab end with and pull until the lidocaine pops out, pull it out of the syringe
 - iii. Carefully and firmly twist to unscrew the needle (if cover comes off, scoop it up with the needle and push against counter)
 - iv. Place lidocaine and needle in biohazard waste bin
 - c. Place the rest of the tools, plastic mirror, wells, rubber dam frame and forceps, clamp and bite block (take off floss) on the tray
 - d. (Leave the curing light, flowable, bottle of Equia Forte coat, etch/sealant/ViscoStat in the bay and clean when you come back from cleaning the tray)
2. Take handpieces off the unit and take burs out of the handpiece

- a. While pinching the bur and pulling out, press the circular face of the bur inwards
 - b. If it's stuck, put it aside near the sink
3. Take barriers (and tips) off the curing light, etch, sealant, ViscoStat, Ultra-blend
4. Take the tray, tools, etc. to the sink and ultrasonic cleaner
 - a. Scrub/rinse off residue on wells, burs, clamp, and other tools if necessary
 - b. Put everything except handpieces in the ultrasonic (burs in cup). Run for 15 min. (Place handpieces aside to put in the autoclave)
 - c. Scrub, spray, and leave tray to dry
5. Remove contaminated gloves and put on new ones to clean
6. Take off used paper and put the mixing pad back. Wipe glass mixing block.
7. Wipe bottles (bond, SDF, Equia Forte coat, cement, IRM), curing light, flowable and Metapix with cavicide if touched during treatment and put back (also replace flowable and Metapix tips if used and put back in respective drawers)
8. Wipe etch, sealant, ViscoStat, Ultra-blend syringes. Put new tips and syringe covers.
 - a. Sealant & Ultra-blend (red brush), etch (round/blue tip), ViscoStat (round/brown tip)
 - b. Cut or pull off excess plastic on the end of the cover
 - c. Refill if there's not enough left, don't throw away
9. Clean room/bay

Sterilizing Instruments

1. Set aside handpieces, prophylaxis angles, and Cavitrons (ultrasonic scalers)
 - a. DO NOT put handpieces in the ultrasonic cleaner. Sterilize them using the autoclave only. Scrub dry, wet a damp napkin/towel with warm water and wipe if needed,.....?
 - b. Submerging (e.g., putting them in an ultrasonic cleaner), running them under water, or wiping/spraying with chemical disinfectants can damage them
2. Ultrasonic cleaner (aka the BioSonic)
 - a. Scrub prophylaxis paste, blood, fillings, cement, etc. off instruments and put into basket
 - b. Put smaller pieces (e.g., cavitron tips, burs) in the cup, and put the cup into the basket
 - c. Place the basket in the ultrasonic cleaner (make sure tools aren't caught in the holes)
 - d. Sterilize instruments/tools (EXCEPT handpieces) in the ultrasonic cleaner for **15 min**
 - e. Take the basket out and run tools under water to rinse off before emptying on top of a towel to dry
3. Bag tools and instruments in pouches to put in the autoclave (check drawers for how they should be arranged in the pouch)
 - a. Explorer tips should be on the opposite side of mirrors to make sure they don't scratch the mirrors
 - b. It's helpful if size labels on bite blocks and rubber dam clamps (e.g., 8A, Child size) are visible/facing up
 - c. Rubber dam frames and clamp forceps are stored together, in the same pouch
 - d. Restorative kits should have:
 - i. **1 bite paper holder/forceps**
 - ii. **1-2 tweezers**
 - iii. **~ 9 tools (with same colored bands)**
 - iv. If the restorative kits have broken tools or are missing important tools, keep the kit together in a pouch and put it on the side
 - e. Spatula and one tweezers go together

- f. Any extra tweezers or bite paper holders can go in their own pouch
- 4. Autoclave
 - a. Put the pouches in the autoclave face down for better ventilation while drying
 - b. Check the water level and refill with distilled water if needed
 - i. Top: check that water is filled up to the green area
 - ii. Bottom: make sure water is filled up to at least halfway before running each time
 - c. **Pouches (“Wrapped” for the larger autoclave) > Start**
 - d. You can also end the cycle while they are drying, if in a rush
- 5. Always check that instruments are sterilized before putting back in the corner of the pouch:
 - a. pink = unsterilized, gray = sterilized

Cleaning the X-ray Room

- 1. Wipe with caviocide: sensors/cords, camera, chair (you don’t have to wipe down the chair if using again with the patient’s family members), also apron if saliva or sensor fell on it
- 2. Replace barriers on the sensors, button (and mouse if you touched it)

Dentrix (Appointment Book, Patient Chart, Family File, Ledger)

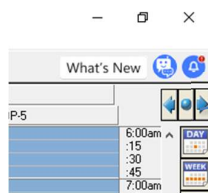
****Due to HIPPA, if the computer is in a highly visible place, minimize the window with chart and appointment book so patients' personal/health information is not visible (unless it's their own chart)****

To start: Open the Appointment Book , then Imaging Bridge  icon on the desktop screen or taskbar to open (move the Imaging Bridge away from the X (close window) button).

Appointment Book

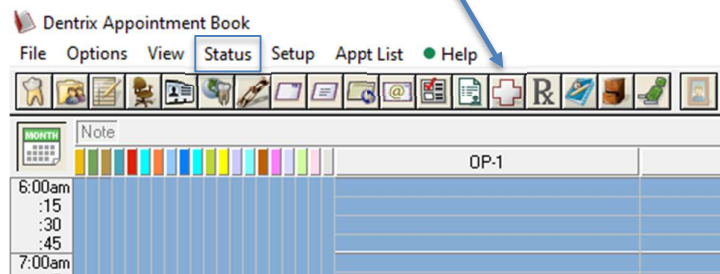
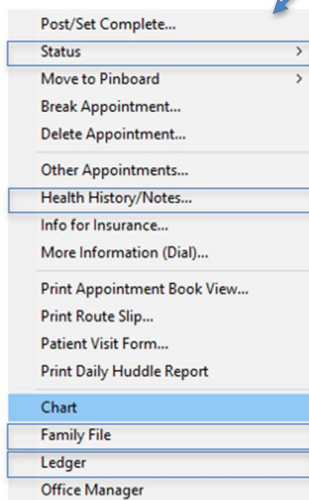
Appointment blocks are color-coded according to provider (doctor and insurance):

- Doc initials, last digit 1 (AA1, A1C1, A1B1) = HDS and other private insurance
- Doc initials, last digit 2 (AA2, A1C2, A1B2) = Quest
- Doc initials, last digit 3 (AA3, A1C3, A1B3) = HMSA and United Concordia



Arrow keys to go to the previous or next day.
Circle to go to the current date.

- Double-click the appointment block to expand notes, check coverage, see provider/insurance
- Hover over appointments to see the patient's age, name, gender, provider, etc.
- To open the chart, family file, or ledger, click on an appointment, then:
 - Right click on appointment for more options OR click on an icon:



-  **Chart** (to add to the ledger, see clinical notes, treatment plan, etc.)
-  **Family File** (to see more info about patient, family members, insurance, etc.)
-  **Ledger** (check if they paid for nitrous, if the ledger was printed, etc.)
- For Limited Evals, double-click on appointment or open "Health History/Other Notes"

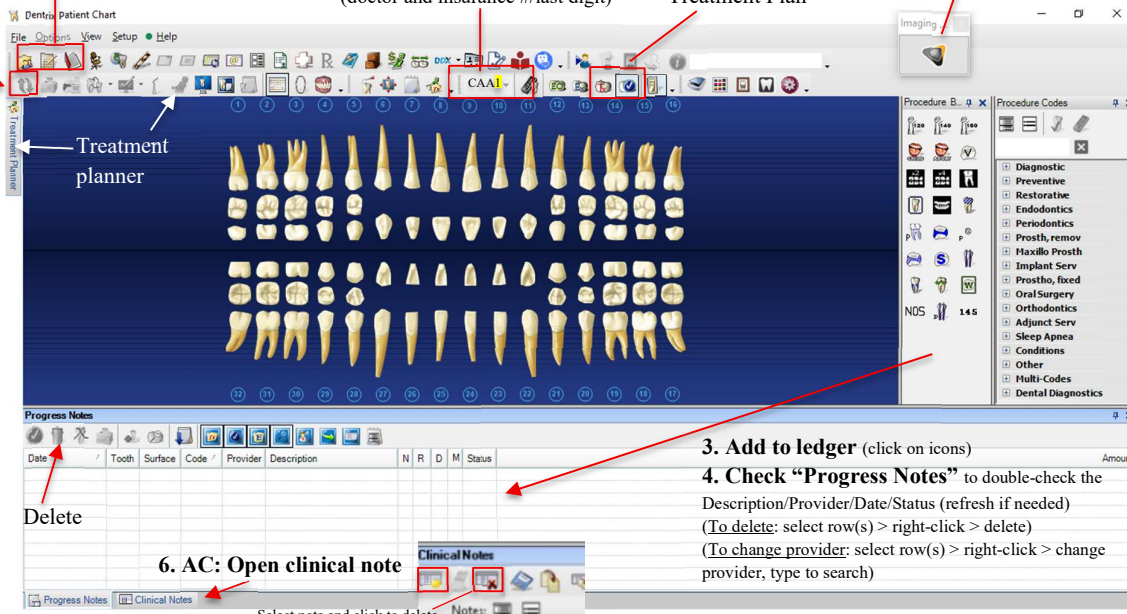
- **Status** (check if patient is “**Here**” (blue) or “**Ready**” (purple) AND change to “**Treatment in Progress**” (green) once you bring them in)

For new patients, wait until the front desk inputs info to look up patients by name

- Click on the New Patient’s appointment or any blank cell. Click on the  **Chart** icon at the top of the appointment book and type their name.

Patient Chart

How to “open”/set up chart in a room:



Refresh (progress/clinical notes, treatment plan)
Family file, ledger, appt book

1. Change to correct provider (doctor and insurance #/last digit)

2. Before adding to ledger, make sure **Completed**  is selected, not Treatment Plan

3. Add to ledger (click on icons)

4. Check “Progress Notes” to double-check the Description/Provider/Date/Status (refresh if needed)
(To delete: select row(s) > right-click > delete)
(To change provider: select row(s) > right-click > change provider, type to search)

5. Open x-ray

6. AC: Open clinical note

Treatment planner

Progress Notes

Delete

Clinical Notes

Select note and click to delete

Procedure Codes

Diagnostic
Preventive
Restorative
Endodontics
Periodontics
Prosth, remov
Maxillo Prosth
Implant Serv
Prosth, fixed
Oral Surgery
Orthodontics
Adjunct Serv
Sleep Apnea
Conditions
Other
Multi-Codes
Dental Diagnostics

If a “Health History Alert” or “Note” pops up, skim and click “OK” or X

Teeth

- Labeled by numbers for adult teeth and letters for baby teeth
- Surfaces: O = occlusal, DO = distal occlusal, MO = mesial occlusal, LO = lingual occlusal
- Red = needs restoration
- Blue = restoration completed
- Black circle + arrow = teeth are shifting

Abbreviations for clinical notes

- NV = next visit
- LC = lidocaine; LA = local anaesthetic
- NO = nitrous oxide
- Be or beh = behavior (1 = very difficult; 4 = well-behaved)
- Comp = resin composite (likely flowable, maybe packable)

Filling out the Ledger

Example for prophy:

- 1) Hover over icons to see the label/procedure name, click on icons to add to ledger

1) Periodic Oral Exam (6-month recall)
OR Oral Eval Under 3 (6-month recall, 0-2 years-old)
OR Comprehensive Oral Exam (all new patients)

2) Prophylaxis-child OR Prophylaxis-adult
(1: 13+ years, 2: 15+ years, 3: 14+ years)

3) Topical Fluoride Varnish

4) Bitewings-two film (12-)
OR Bitewings-four film (13+)

5) Intraoral-occlusal (1x for Quest)

Panoramic (if taken; not for pano bitewings)

5) Intraoral-periapical-1st film / each
addtl (1x for Quest, 2x for private)

- 2) Go to “Progress Notes” (refresh if necessary)
- 3) Double-check that the **descriptions**, **provider** (doctor and insurance), and **status** [C(ompleted) or T(reatment) P(lan)] are correct and that they are under the **current date**
 - a. Example of a Prophylaxis, 6-month recall/periodic oral exam, with Dr. Todd Asato and private insurance:
 - i. **2 intraoral-periapical-1st films for private**
 - ii. **1 intraoral-occlusal and 1 intraoral-periapical-1st film for Quest**

Progress Notes										
										
Date	Tooth	Surface	Code	Provider	Description	N	R	D	M	Status
12/25/2025			D0120	T1A1	Periodic oral exam					C
12/25/2025			D0230	T1A1	Intraoral-periapical-1 st film					C
12/25/2025			D0230	T1A1	Intraoral-periapical-each addtl					C
12/25/2025			D0272	T1A1	Bitewings-two film					C
12/25/2025			D1120	T1A1	Prophylaxis-child					C

For Reference:



Stainless steel crown



Pulp



Resin



Amalgam



?



Sealant



Extraction/erupted tooth



Root canal



Fractured tooth



Watch tooth



Nitrous oxide



Pulp + extraction/erupted tooth

Summary of Ledger Items

Prophylaxis (Prophy) Or Comprehensive Oral Exam (CompEx)	6-month recall:  Periodic Oral Exam	
	6-month recall for 0–2-year-olds:  Oral Eval Under 3	
	New patients:  Comprehensive Oral Exam	
	Depending on provider: <table><tr><td> Prophylaxis-Child If the last digit is: 1 – 12 and under 2 – 14 and under 3 – 13 and under</td><td> Prophylaxis-Adult 1 – 13+ years 2 – 15+ years 3 – 14+ years</td></tr></table>	 Prophylaxis-Child If the last digit is: 1 – 12 and under 2 – 14 and under 3 – 13 and under
 Prophylaxis-Child If the last digit is: 1 – 12 and under 2 – 14 and under 3 – 13 and under	 Prophylaxis-Adult 1 – 13+ years 2 – 15+ years 3 – 14+ years	
 Topical Fluoride varnish Usually, no x-rays for 0–3-year-olds 12y and under:  Bitewings-two film 13+:  Bitewings-four film <i>Still select bitewings-two or four-film if taking pano bitewings (not Panoramic)</i>		
	For Quest (last digit 2):  Intraoral-periapical-1st film +  Intraoral-occlusal film For private insurance (last digit 1 or 3):  Intraoral-periapical-1st film +  Intraoral-periapical-each additional Patients 9+ that have not taken pano yet:  Panoramic	
	Limited Oral Evaluation  If an x-ray is taken:  Intraoral-periapical-1st film	
	• Open “Treatment Planner” (make sure correct provider/doctor is selected) • Ctrl + select completed restoratives • Right-click > “Set Complete”	

Behavior Management

Handling difficult patients

- Ask them to keep their hands on their tummy or to hold or squeeze your hands
- Ask them to take deep breaths or visualize calming scenarios
- Hold their arms and/or legs down
- Offer encouragement
- Work quickly
-

Rules for Patients and Family

- No running around; patients and their families must stay in their room
- Parents should not be standing in the hallway or anywhere outside of the room their child will receive treatment in, unless they are helping to manage their child's behavior.
 - Ask parents if they would like to have a seat in a room
 - If they decline, let them know it is unsafe to be standing in front of the x-ray room or in the way, or wherever they are.
- ...

Bringing patients in

1. Make sure the room and tray is set up
 - a. If you have time, make goody bags (for prophylaxis appointments).
2. Check the age and open charts of all expected appointments in the same time slot
 - a. Check if there are any siblings or families with appointments
 - i. If there are, try to put them in the larger rooms
 - ii. If the middle (smallest room) is open, prioritize putting patients who are coming in alone (without parents or siblings) there first.
 - b. Check notes on behavior
 - i. Consider putting difficult patients in the Fish room (with doors)
 - ii. AC — behavior ratings in clinical notes: Beh = behavior (1 = very difficult; 4 = easy)
3. Check status
 - a. Wait until patients are “Ready” (purple) to bring them in or if it’s been 5 min, check with the front
 - b. For patients with Quest insurance, bring them in once they are “Here” (blue):
 - i. If old enough to go by themselves, while their parent finishes their paperwork
 - ii. Or wait until patient finishes their paperwork
 - iii. (With Quest, everything for a periodic exam will usually be covered)
 - c. If it’s been awhile, check if the front desk is still processing forms or forgot to change their status
 - d. If a patient is ready but there are no more rooms available, take their x-rays and send them back to the waiting room until a room becomes available.
 - e. After 15 min, it’s a no-show
4. Make sure the right patient’s chart is pulled up in the room
5. Check their file and take it to the back with you
 - a. Before you call them, check the first page of their file (e.g., no pano, no x-rays, no fluoride)
6. Call the patient and take them to get x-rays or bring them to the chair
 - a. If there is a family/siblings, usually:
 - i. Bring them in together and put them in the same room (try not to put them in the middle room since there’s not much space)
 - b. Don’t let parents stand in the hallway (show them to a room)
7. Take turns bringing in patients
8. Take x-rays and assist for the patients you bring in or if no one else is there to assist

Types of Appointments

- Prophylaxis (prophy) — cleaning, x-ray
- Comprehensive oral exam (CompEx) – checkup + prophylaxis for new patients
 - Set up: prophylaxis tray, pamphlet for 1-year olds (*Your Child's Teeth*, back counter)
- Limited exam
 - have mirror/explorer ready, read note
 - ask what the problem is and which tooth, take x-rays on affected area(s)
 - If they've taken X-rays at a different office and we retake them, there is no charge
- Emergency exam
 - have mirror/explorer ready, read notes, take x-rays on affected areas
- Treatments
 - Resin Composite (ResComp)
 - Glass ionomer (Equia/Riva)
 - Flowable
 - Packable
 - Amalgam
 - Stainless Steel Crown (SSC or crown)
 - Sealants (Seal)
 - Extraction (Ext)
 - Pulpectomy (P or Pulp)
 - Fluoride or Silver Diamine Fluoride (SDF)

Types of Restoratives

- White filling = resin composite (e.g., flowable, Riva, Equia, packable)
- Silver filling = amalgam
- Sealant = covers the fissures in the tooth to prevent decay
- Pulpectomy = removing the nerve/the top part of the root
- Stainless steel crown = covers the tooth after it's shaved down, when it's not possible to restore with a filling

Radiographs (x-rays)

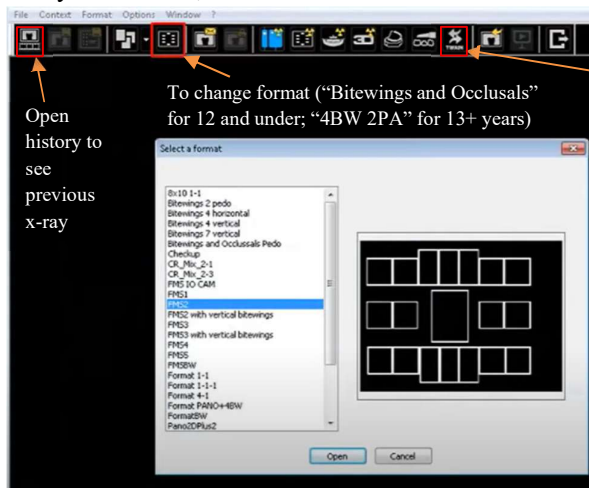
- Help us look inside teeth for problems invisible to the naked eye, can often diagnose cavities much more quickly with an X-ray
- Helpful for finding any gum or teeth problems (Source: <https://keikismiles.com/services/digital-x-rays/>)

Periapical & Bitewing (for prophys, 6-month recall, or new patients)

1. Do you need to take an x-ray and which type?
 - 0-3 years old: usually no x-rays
 - 4-12 years: 4 x-rays in total
 - 2 PAs (periapical) – anterior, top & bottom
 - 2 BWs (bitewing) – molars, left & right
 - 13+ years: 6 x-rays in total
 - 2 PAs – anterior, top/bottom
 - 4 BW – 2 left & 2 right (molars and premolars on each side)
 - 9+ years: panoramic x-rays if none taken before
2. Before/when you bring the patient in, check the patient's file/folder to see if x-rays are covered by insurance (the front desk will write it on the first page of the patient file)
 - Note: "No BWs" => don't take any X-rays including PAs
3. Ask parent if it is okay to take x-rays
4. If there is time, open the patient chart, fill out the ledger, and open x-ray before bringing in the patient
 - Open chart > click on Imaging Bridge (or click on desktop icon for first-time use)



5. Double-check that the patient name is correct
6. Make sure to have the correct format for the patient's age: select "Bitewings and Occlusals" (for 12 years & under) or "4BW 2PA" (for 13+ years)
 - 4-12 years: 2 PAs (periapical), 2 BWs (bitewing)
 - 13+ years: 2 PAs, 4 BWs



To take pano: press "TWAIN", it should say "Ready"

- <https://www.youtube.com/watch?v=N3sx2xdbwLA>
7. Before starting, make sure to click the frame that corresponds to the teeth you are capturing, and that the frame is outlined in green (whichever frame is green, is where the image you're capturing will go)
 - Alternatively, after taking the x-ray, click and drag the image to the correct frame
 - It should look as if the patient is facing you (e.g., top front teeth should be in the top middle frame)
 8. Put on gloves
 9. Put apron on patient (select the appropriate apron size)
 10. Take x-rays
 - Periapical
 - i. For the anterior, place sensor in mouth facing up (for maxillary) or down (for mandibular)
 - ii. Sensor should be centered btwn the upper canines and lateral incisors
 - iii. Angle camera towards nose (for maxillary) or chin (for mandibular)
 - iv. Make sure the teeth do not extend beyond the sensor and that it is not positioned too far back or forward in the mouth
 - Bitewing (two for ages 12 and under, four for 13+)
 - i. Posterior – sensor in back teeth, behind back molar showing
 - ii. If taking 4 bitewings, make sure to get premolars on one image
 - iii. There should not be overlap between teeth in the image (aim camera at a perpendicular right angle to the sensor on a flat plane, or use positioning device)
 - iv. Can also change the sensor holder/use a larger sensor for older patients
 - v. If 4- or 5-year-olds cannot take bitewings, it's okay to just do PAs.
 - vi. If it's too difficult to take x-rays using the intraoral camera (e.g., b/c of gag reflex), try taking panoramic bitewings instead (not to be confused with regular panoramic)
 - Check that no one is passing by the room (if there is, wait until they pass by the door or ask them to wait behind the door), then press the button on the wall outside the room
 - Check the x-ray and make adjustments for the next one if needed
 - Do the next side. Take off the barriers once you're done using them. Finish all sides.
 - Make sure the images are in the correct frames to match the side they were taken on. Click and drag the image to the correct frame if necessary.
 - Flip images of anterior teeth so the teeth are facing the center/inwards
 11. Escort the patient to a bay and open the x-rays and/or chart in their bay, if you haven't done so already
 12. Clean the room
 - Wipe sensors, camera, and chair (don't have to wipe down chair if you're taking their siblings' next)
 - Put barriers on sensors
 - Replace barriers on button (and mouse if you touched it)

How to Adjust the Image (orientation, saturation, etc.)

- ...

Moving X-rays

- If you want to move the x-rays to a different patient file:
 - Select images > “Send to” > File location: desktop
 - Open the patient you want to move the images to > click and drag files from the desktop
 - Select images > “File” > “delete”
- If you want to change the number of BWs:
 - Select images > “Send to” > File location: desktop
 - Change from “Bitewings and Occlusals” to “4BW 2PA” or vice versa
 - Click and drag files from the desktop

Panoramic

- Usually, the doctor will ask you for a pano (e.g., to check wisdom teeth)
- Also used if normal x-rays are difficult (change settings for different types of panos)
 - Child has gag reflex or is difficult
 - Read notes

Steps

1. Have gloves on
2. Set up the panoramic X-ray machine
 - a. On the computer: press Twain
 - b. Adjust height
 - c. Reset
 - d. Place barrier on mouth piece
 - e. Select the correct type of pano (e.g., pano bitewings)
3. Set up the patient
 - a. Ask them to take off jewelery (earrings, necklaces)
 - b. Make sure the vest is properly fitted on the patient and aligned in the back (otherwise the vest shows up in the x-ray)
4. Ask them to step forward, put their chin on the ___, and have their teeth between the groove
5. Line up lasers (+)
 - a. To adjust vertical laser, adjust headgear
 - b. To adjust horizontal laser, use lever on the machine
6. You can ask them to hold onto the sides
7. Make sure their face is relaxed (if not, it will look like the patient is smiling in the pano)
 - a. Make sure they aren't slouching and their head isn't raised
 - b. If they are younger, you can also remind them to stay still
8. Hold down on pano button (on the wall outside the room) until sound ends
9. Adjust photo
 - a. Don't forget to flip x-ray photo as if doc is looking at patient
 - b. You can also adjust the brightness, contrast, etc.
10. Take off vest and send patient back to the room or waiting room

- a. AC – If doing x-rays after cleaning: “The doctor will call you back if she sees anything. If you don’t get a call back, don’t worry. It just means everything looked normal.”
- 11. Clean
 - a. Take barrier off mouthpiece
 - b. Wipe everything down
- 12. Reset and raise the machine

Other (e.g., limited oral evaluations)

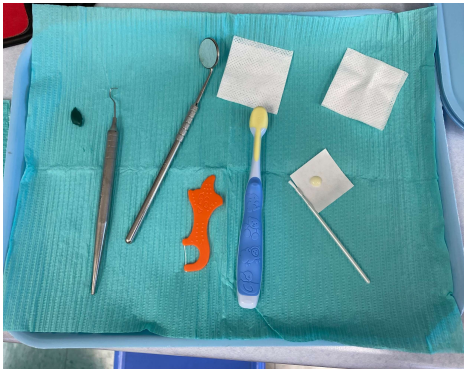
Suctioning

Prophylaxis (Prophy) – Cleaning

Set up

For every prophy:

- Mirror and explorer
- Prophylaxis paste
 - Take the explorer and swipe a bit of prophylaxis paste from the container. If you need more use the end of a Q-tip to get it to avoid “double-dipping.”
 - (Exactly how much?) Put at least ~1/3 to 1/4 the container for older patients, for ages 0-3 only a small amount is needed
 - Try to only use mint flavor on teenagers & adults since it might be too spicy for young children
- Floss
 - ~ 1 ft of string floss or a pick for 3 and younger
- A drop of fluoride and brush
- AC – add two 2x2s
 - May need periodontal probe and scalers for 17+ year olds – top drawer near autoclave
 - For patients with fixed retainers, have “ortho floss” (floss threader) – in the bay
 - For patients with braces, have tapered prophylaxis brush and “ortho floss pick”
- For ages 0-3: Add floss pick instead of string floss, toothbrush, 2 gauze/2x2s



Steps

1. Bring the patient in (see “Bringing Patients in”)
2. Take their x-rays (see “X-rays”)
3. Take them to a room
 - a. AC – put patient file on the table or near computer; AB – put files on the back counter
 - b. Make sure chart and their most recent x-rays are open (let the doctor know if bitewings were not covered by insurance)
 - c. Make sure ledger is filled out
 - d. AC – open a new “clinical note” on the chart
4. Clean the x-ray room (if the doctor is not ready yet)
5. Wait for the doctor
 - a. Talk to patient, ask what they want to watch on the TV, check the tray and room
 - b. Have goody bag(s) ready
 - c. Help clean rooms
 - d. Empty the Biosonic (and rinse tools) and/or bag instruments to put in the autoclave
 - e. Put back sterilized instruments from the autoclave
 - f. Wipe drying trays with alcohol and put back in the cabinet
 - g. Restock
 - h. Prep treatment trays for the day or next day
 - i. If there seems to be a lot of time, take x-rays and prepare other patients
6. Put on face shield and gloves
7. Once the doctor lowers the chair, place a napkin on the patient’s chest
8. AC — have sunglasses ready
9. Lower the light
 - a. Be careful not to shine the light in their eyes, direct it towards their lower jaw and adjust as needed throughout
10. Make sure that the tray and bay are set up properly, for example:
 - a. Floss, prophylactic paste, fluoride, brush, (AC – 2x2s)
 - b. Cavitron tip and prophylactic brush
11. Get ready to use saliva ejector (bend it)
 - a. Suction saliva pooling in the back, on the sides
 - b. Follow the prophylactic brush to keep prophylactic paste from flying
12. Use the HVE when they use the cavitron
 - a. Suction on opposite surface
 - b. Don’t go too close to the soft tissue (cheek, tongue). If the HVE gets stuck, turn it off and then move it to avoid hurting the patient.
 - c. Don’t block the doctor’s view/mirror
13. If the doctor and parent decide to do an extraction or other restorative, procedure, etc.:
 - a. See notes under “Limited Oral Evaluations” (consent form signed, nitrous paid, etc.)
 - b. Set up for treatment (bring the necessary tools, handpieces, burs, etc., no need to set up another tray specifically for treatment)
14. After cleaning is done, rinse to get rid of prophylactic paste and blood (if necessary)
15. Fluoride
 - a. Apply fluoride, then use saliva ejector to help get rid of taste and saliva
 - b. **Instructions for fluoride:** You can eat or drink right away. It might feel a little sticky throughout the day. Just brush your teeth like normal tonight.

16. If the doctor is still talking to the patient/parent, you can take the tray to the sterilization area and start to clean the bay
 - a. Quickly scrub the tray with brush and dish soap, spray with cavicide on both sides 1x, lean against the wall on the towel to dry
17. Give them their goody bag and offer a sticker and toy
18. Escort the patient out
 - a. Write referral if needed (and make a copy or give to front desk to copy)
 - b. Put their file in either the “Cleaning” or “Treatment” slot depending on whether the doctor added a treatment plan or not AND tell the front desk “6 months” (for cleaning) or “treatment”
19. Talk to parents (for patients that went in alone and are under 18)
 - a. For younger patients: Did well OR let them know if there were any major problems, (e.g., they threw up)
 - b. Explain what the doctor said (e.g., the doctor didn’t find any cavities OR they found a cavity and need to come back to get it filled, etc.)
 - c. Brushing and flossing is good OR (explain what the doctor said)
 - i. E.g., [Name] did well today. The doctor didn’t see any cavities. His/her/their brushing looks good, but they should work on flossing more.
20. Clean the room
 - a. Take the tray (and Cavitron tip if you used it) to the sterilization area and clean if you have not already
 - b. Take off used items in the bay and barriers that were touched
 - c. Wipe down the bay and counter with cavicide
 - d. Wipe used chairs (patient, doctor, assistant, parent chair)
 - e. Replace barriers and straws, prophylaxis brush, and cavitron tip as needed
 - f. Close patient chart and open appointment book
21. Make sure the ledger is filled out

Limited Oral Evaluations

1. Have tray with mirror and explorer ready (have Chart and the last x-rays taken open on computer or open once they are assigned to a room)
2. Bring in patient and folder
3. Ask what the problem is/why they came in
4. Take x-rays if needed
 - a. When to take x-rays?... (examples of when to take x-rays/what to do)
 - b. If they were referred by and took x-rays at another office, and we need to re-take the x-rays, don't add the x-rays to the ledger
5. If they decide to do treatment that day:
 - a. Ask the parent/legal guardian to sign the consent form for the treatment plan (or at least have the doctor and front desk get verbal consent, e.g., over the phone)
 - i. If they have Quest, usually everything will be covered, and you can have them sign the consent forms before they leave
 - ii. If they have private insurance, they must sign the consent forms before the procedure starts
 - b. If using nitrous and their provider is Quest, kindly let them know that their insurance does not cover nitrous oxide and the front desk can help them with the payment before starting the procedure (double-check with the front desk that using nitrous is okay, once they pay)
 - c. Set up for treatment (bring the necessary tools, handpieces, burs, etc., no need to set up another tray specifically for treatment)

Treatments

Before Treatment:

- Before setting up, **check the ledger to see if nitrous is paid for** (if not, double check whether we're using it)
- **Check to see if consent form is signed**
- Check the Treatment Plan, Chart, and Clinical Notes
 - Treatment Plan – what type of treatment
 - Resin Composite (ResComp)
 - Glass ionomer (Equia or Riva)
 - Flowable
 - Packable
 - Amalgam
 - Stainless Steel Crown (ssc or crown)
 - Sealants (Seal)
 - Extraction (Ext)
 - Pulpectomy (P or Pulp)
 - Chart
 - Which teeth? (what shade of Riva/Equia/flowable, which forceps for extractions, how much cotton rolls and gauze/2x2s)
 - Which surface? (need matrix bands and wedges for decay between teeth)
 - Clinical Notes
 - patient behavior, do we need a papoose, etc.
 - NV = next visit; LC = lidocaine; NO = nitrous oxide; LA = local anaesthetic, comp = resin composite (flowable)
- Set up the treatment tray
 - Set up in any room with oxygen/gas tanks, “Fish room” for difficult patients
- When bringing the patient in, **check parent signature** and whether the treatment will be **with or w/out nitrous**

Setting up for Restoratives and Other Treatments

Check the Chart

1. What is their age?
 - a. What size bite block (child or adult)
 - b. Behavior, which room to use, is a mouth prop needed
 - c. Which clamp size (8A or 14A) to use, if using the rubber dam
 - d. AC: which needle to use when
2. Which procedure is it?
 - a. Check notes for specific details (NV: next visit) and behavior
 - b. Ask doctor if notes have insufficient details
3. Is nitrous oxide being used? (and if they have Quest, check the ledger to see if it's paid for)
4. Which teeth?
 - a. Adult or baby teeth?
 - i. If using a rubber dam:
 1. Is the patient younger (under what age?)?
 - a. Use a clamp for baby teeth (8A, 14, 7)
 2. Is the last or second to last tooth in the back and adult tooth?
 - a. Use a clamp for adult teeth (14A)
 - ii. If doing a white filling, use:
 1. A1 for front adult teeth,
 2. A2 for back adult teeth,
 3. or B1 for baby teeth
 - b. Is the decay/cavity on the mesial/distal (side) surfaces?
 - i. Matrix band(s) (1 for each tooth with decay on the mesial/distal surfaces)
 - ii. Wedge(s) (AA, AB: 2 per interproximal space, AC: 1)
 - iii. AC: Is the patient a teenager/adult?
 1. Garrison box with garrison clamps and wedges instead of regular
 2. (sandpaper) polishing strip
 3. “#12” = Blade, scalpel handle, needle holder
 - c. Good to keep in mind during the procedure to make sure all the correct teeth have been worked on

Find a tray

1. Appointment scheduled for treatment: get a clean treatment tray
2. During prophylaxis/exam: add to prophylaxis tray

Gather necessary pouches of tools from drawers

1. For every treatment besides prophylaxis (i.e., restorative, sealant, or extraction), have:
 - a. Mirror and explorer
 - b. Bite block (and have mouth prop unopened for younger/more difficult patients)
 - c. Aspirating syringe (<https://www.youtube.com/watch?v=DLbYOwlwvcY>)
2. Which type of treatment?
 - a. Extraction
 - i. Forceps

- 1 = anterior (front) teeth
 - 150 = maxillary (top) molars
 - 150s = top baby teeth
 - 151 = mandibular (bottom) molars
 - 151s = bottom baby teeth
 - ii. Elevator
 - Periosteal elevator for anterior teeth
 - Straight elevator for molars
 - b. Sealant:
 - i. High-speed handpiece
 - ii. Bite paper holder
 - c. Restorative (Riva, Equia, flowable, packable, pulp, crown):
 - i. Restorative kit
 - 1. (make sure important tools are not missing or broken, e.g., Hollenback carver for glass ionomer)
 - ii. High and low-speed handpieces
 - iii. Rubber dam frame and forceps for clamp (or forceps for garrison clamp)
 - iv. Clamp (baby teeth: 8A, also 14, 7 | adult: 14A)
3. For restoratives, which type of restorative?
- a. Glass ionomer (Riva and Equia):
 - i. Applicator
 - b. Packable:
 - i. Packable gun
 - c. Crown:
 - i. Spatula and tweezers
 - ii. Howe pliers
 - iii. Bite stick
 - iv. Crown crimper (does not have a hole in the tip unlike the wire crimper)

Put the burs on the tray

- 1. What type of treatment/procedure is it?
 - a. Extraction no burs
 - b. Sealant.....round white stone bur
 - c. Restorative..... 34SS, 557, 4SS, 6SS (AC - 330)
- 2. If it's a restorative, what type of restorative?
 - a. White filling..... add FB gold, gold, stone (round & pointy), polish disk
 - b. Silver filling..... nothing to add
 - c. Pulp and/or crown..... add FB flame (round & pointy)

Set up the tray

- Have 2 napkins: 1 napkin next to the tray to catch spills and 1 for the patient
- Open pouches and assemble tools on the tray (and on the napkin; can organize according to order of use)

Ligate/tie floss onto anything that could fall down throat (bite block, clamp)

- Instructions and picture here

Gather other materials

- Extraction
 - 2 folded 2x2 gauze per extracted tooth to stop bleeding
 - 1 unfolded 2x2 gauze to catch tooth in case it falls
 - If using local anaesthetic: syringe, lidocaine, needle
 - AA, AC: topical anaesthetic (swipe a bit from the container using a Q-tip and place on 2x2 gauze)
 - How much?/when to put more than one
 - Treasure box
 - Cup with extra gauze
- Sealant
 - Etch (if there's not much left in the tube, have an extra)
 - Sealant (if there's not much left in the tube, have an extra)
 - Bond, well, bond brush (wait until starting to add a drop of bond, keep the bond covered b/c it's light sensitive)
 - Curing light with barrier
 - Extra mirror
 - Cotton rolls and 2x2 gauze
 - Maxillary (top) – 1 cotton roll on each side (i.e., left or right side)
 - Mandibular (bottom) – 2 cotton rolls on each side, 1 wrapped in gauze
- For all restoratives...
 - All except crowns: Rubber dam sheet (unless using mouth prop or cotton rolls)
 - AC: set up (but don't have the clamp in the forceps to avoid stretching it out)
 - AA: open the pouch and have dental dam sheet ready, but don't assemble it
 - AB: don't open the pouch, have on the side
 - If using local anaesthetic: lidocaine and needle
 - AA, AC: topical anaesthetic (swipe a bit from the container using a Q-tip and place on 2x2 gauze; How much?/when to put more than one)
 - If decay is on mesial/distal (side/between) surfaces, have matrix or garrison bands and wedges
 - Matrix band(s) (1 for each tooth with decay on the mesial/distal surfaces)
 - Wedge(s) (AA, AB: 2 per interproximal space, AC: 1)
 - AC: Is the patient a teenager or adult?
 - Get Garrison box with garrison clamps and wedges instead
 - (sandpaper) polishing strip
 - “#12” = Blade, scalpel handle, needle holder



- For specific restoratives...
 - Glass ionomer
 - Curing light (cover with barrier)
 - Well
 - Wet Q-tip or wet micro-brush (can bend brush tips before starting)
 - Bite paper (rip off a sheet to put into forceps/holder)
 - Riva or Equia Forte?
 - Riva: 2 Riva capsules (A1 for front teeth, A2 for back teeth)
 - Equia: 2 Equia Forte fil capsules (A1 for front adult teeth, A2 for back adult teeth) + bottle of Equia Forte coat
 - Cavity conditioner + micro-brush (AA – etch)
 - Pour out cavity conditioner (blue liquid) and Equia forte coat (yellow liquid). Cover with bottle since it's light sensitive or wait to pour it out) in the well. Open 1 package containing Riva or Equia capsule to have ready for mixing.
 - Resin composite
 - Curing light (cover with barrier)
 - Etch
 - Well
 - Bond and bond brush (can bend brush tips before starting)
 - Micro-brush (can bend brush tips before starting)
 - Bite paper (rip off a sheet to put into forceps/holder)
 - Flowable or packable?
 - Flowable: Flowable (A1 for front adult teeth, A2 for back adult teeth, or B1 for baby teeth)
 - Note: if there's not much left in the tube, have an extra
 - Packable: Capsule (A1 for front adult teeth, A2 for back adult teeth, or B1 for baby teeth), put into packable gun
 - Amalgam
 - Amalgam carrier
 - Amalgam well
 - Q-tip
 - Pulp and crown
 - Pulp and crown or just crown?
 - Pulp and crown: IRM + glass block, Viscostat
 - IRM: for 1 tooth, 1 scoop and 1 drop
 - Crown: SDF + micro-brush
 - Cement + non-absorbent paper
 - Follow directions on bottle for # of drops per scoops
 - 1 big scoop → 3 drops OR 1 small scoop → 1 drop
 - can add more drops as needed
 - Box of crowns (set 1 tweezers on the box, for use in the box only)
 - Gauze/2x2s and cotton rolls
 - Maxillary (top) – 1 cotton roll on each side (i.e., left, right)
 - Mandibular (bottom) – 2 cotton rolls on each side, 1 wrapped in gauze

Take off Cavitron and attach handpieces (can also test handpieces)

- How to test handpieces

Check if using nitrous oxide and set up

1. Do they have Quest as their insurance/provider?
 - a. Yes.....check if it's been paid for in the ledger since after the last treatment (~\$83)
2. If they have Quest and it has not been paid for:
 - a. Kindly let them know their insurance does not cover nitrous oxide and that the front desk can help them
 - b. Afterwards, check with the front desk that it's okay to use nitrous
3. Set up
 - a. Attach nose
 - b. Are the tanks empty?
 - c. Barrier on gas/oxygen tanks
 - d. Connect scavenging circuit (pipe)

Prophy**0-3 years**

- Add floss pick instead of string floss, toothbrush, 2 gauze/2x2s

**4 and older**

- Mirror and explorer
- Prophy paste (~1/3 or half container if older)
- Floss (~1 ft of string floss)
- Fluoride and brush
- AC – add two 2x2s (Adults: Periodontal probe or scalers - Fixed retainer: “ortho floss” (floss threader) - Braces: tapered prophy brush and ortho floss pick)

**Extraction**

- Mirror and explorer
- Bite block
- Periosteal elevator (for anterior)
- Straight elevator (for molars)
- Forceps: 1 = anterior (front) teeth, 150 = maxillary (top) molars, 150s = top baby teeth, 151 = mandibular (bottom) molars, 151s = bottom baby teeth
- Syringe (lidocaine and needle)
- Topical anaesthetic (AC, AA)
- 2 folded gauze/2x2 per extracted tooth + 1 unfolded
- Cup with extra gauze and treasure box

**Sealant**

- Mirror and explorer + extra mirror
- Bite block
- Have bite paper holder on the side
- High speed handpiece
- Burs: stone (round)
- Etch
- Sealant
- Bond, well, bond brush (keep the bond covered b/c it's light sensitive)
- Microbrush
- Gauze/2x2 and cotton rolls
- Curing light with barrier

**Pulpectomy + stainless steel crown**

- Mirror/explorer
- Restorative kit
- Syringe (lidocaine & needle)
- Bite block
- Rubber dam and clamp (AC: set up, AA: don't set up, AB: have on the side/don't open)
- Bite paper holder and bite paper
- Howe pliers
- Crown crimper
- Spatula (and tweezers)
- Bite stick
- Cement (for SSC)
- IRM (for pulp)
- Viscostat (for pulp)
- SDF + microbrush (if only doing SSC)
- Gauze/2x2s and cotton rolls
- Topical anaesthetic on cue tip and gauze



- Box of crowns
- Burs: 34SS, 557, 4SS, 6SS, (AC - 330), FB, flame

Fillings

- Restorative kit (White filling: Hollenback | Amalgam: condenser, ball-burnisher, carver, acorn)
 - Mirror and explorer
 - Bite block
 - Rubber dam, clamp (baby teeth: 8A, adult: 14A), forceps/frame (AC: set up, AA: don't set up, AB: don't open)
 - Syringe, lidocaine, needle (AC – check if using in notes)
 - Bite paper + holder
 - Handpieces
 - Topical anaesthetic (AC, AA)
 - Matrix bands & wedges, if decay is btwn teeth (AA/AB: 2 wedges per | AC: 1 wedge per, garrison for teenagers & adults)
 - Gauze/2x2 (~4 pieces)
 - Extra piece of floss
 - Nitrous nose and barrier for gas tanks (check if using and if it was paid for in the ledger)
 - Curing light with barrier (for white fillings)
 - Mouth prop for younger children (have unopened on the side)
 - Burs: 34SS, 557, 4SS, 6SS (AC - 330), **White fillings: FB gold, gold, stone (round & pointy), polish disk**
- Shades of Riva/Equia/flowable/packable – A1: front adult teeth, A2: back adult teeth, B1: baby teeth

Glass Ionomer

- Applicator
- Well
- Cavity conditioner + microbrush (AA – etch)
- Extra microbrush
- Wet Q-tip (or microbrush)

Riva

- 2 capsules of Riva

**Equia**

- 2 capsules of Equia Forte Fil
- Bottle of Equia Forte coat

**Resin Composite (Flowable & packable)**

- Well
- Bond and bond brush
- Microbrush
- Etch (if there's not much left in the tube, have extra)
- Flowable OR packable (packable gun + capsule)

**Amalgam (silver filling)**

- Amalgam carrier
- Amalgam well
- Q-tip

Overview of Restorative Procedures

1. Check that parent/legal guardian has signed the consent form (in the physical patient file behind the treatment plan), before treatment if private insurance or before they leave if Quest
2. Check if nitrous is being used, and if the patient has Quest check that it has been paid for recently (in the ledger: ~\$83)
3. Numbing
 - a. Topical anaesthetic (AC, AA)
 - i. While waiting for the doctor to apply it and for it to work (~2 min):
 1. Double check that we have everything
 2. Make sure the garbage can is not full, if it is then push it down
 - b. Nitrous oxide
 - i. If there's time, you can ask what flavor they want (hot pink = bubble gum, light pink = peach, red = strawberry, purple = grape, orange = orange)
 - ii. Doctor usually turns the gas and oxygen tanks on
 - iii. Flip the switch connected to the gas tanks up (when you connect the hose, make sure it's behind the trap)
 - iv. AC — Hold nose while the doctor lifts the patient up
 - c. Local anaesthetic
 - i. Be ready to hold/block their hands (e.g., especially if they are younger, hold put your arm over the patient and hand on the arm of the chair — if they are older, just watch them)
 - ii. Rinse and suction with saliva ejector right after to rinse out bad taste (then, ask if they need more water)
4. Bite block
 - a. Usually, have a child size unless the doctor asks for the adult size, or if the patient is older
5. Rubber dam OR isolate the area OR use mouth prop
 - a. Rubber dam
 - i. Make sure to have the right clamp (8A = baby tooth; 14A = adult tooth)
 - ii. AB — Have rubber dam on the side unopened, usually not necessary
 - iii. AC — pass the clamp and forceps, then rubber dam (set up the rubber dam but not the clamp/forceps beforehand)
 - iv. AA — pass forceps, clamp, then unassembled rubber dam, then help put on the rubber dam
 - b. Isolate the area
 - i. AC — isolate the area with 2x2s and cotton rolls
 1. Maxillary (top) – 1 cotton roll on each side (i.e., left or right side)
 2. Mandibular (bottom) – 2 cotton rolls on each side, 1 wrapped/rolled in gauze
 3. May also use dri-angle
 - ii. AB & AA — using a mirror, either retract the cheek or hold the mirror against the tongue, depending on which side they're working on
6. Prep the tooth – clean and dry (i.e. suction)
 - a. Suction using the HVE (high volume evacuator, i.e. larger suction)
 - i. If not using rubber dam, get into a good position to retract the cheek or protect tongue with a mirror

- ii. Use HVE suction while doctor is removing cavity using handpiece (to catch excess water and debris)
- iii. have the suction nearby but don't block the area they're working on
- iv. after, suction on top of the area
- v. can use air-water syringe if there is lots of debris or blood making it difficult to see the area
- vi. continue to use HVE while they rinse
- vii. AC – Drying the area → suction the tooth/water, then cotton rolls for ~2-3 sec
- b. Generally, hold HVE on the opposite surface of the tooth the doctor is working on
 - i. If working on the side of the mouth nearest to you → suction on the buccal/facial surface (the side that faces the cheek)
 - ii. If on the top side near the doctor, suction on the lingual surface (the side that faces the tongue)
 - iii. If on the bottom side near the doctor, hold the tongue w/ mirror to block and protect it
 - iv. Anterior (front teeth) → generally suction on the opposite side the doctor is working on and try not to block their mirror
- c. Retract their cheek or hold tongue using mirror, if working without the rubber dam
 - i. If working on the patient's left side, angle the mirror down, towards the bottom of their jaw/gumline while pulling their cheek out, making sure the tissue in the area near teeth is stretched out so that the handpieces are unlikely to come into contact with their cheek
 - ii. If working on the patient's right, angle the mirror downwards so the mirror does not rest on the teeth or teeth and hold tongue away from the teeth/area being worked on
- d. Put the saliva ejector (low volume evacuator, i.e., the smaller suction) near the back of their mouth, on the side, if saliva is pooling
- e. Don't lean against the gums or teeth with the suction or mirror b/c it hurts the patient
- f. Air/water syringe
 - i. use both or a little bit of water to clean area → then air to dry the tooth
- g. Keep the doctor's mirror clean
 - i. If the doctor's mirror is fogging up, put suction (any) nearby
 - ii. If debris is covering the mirror, put the air/water syringe close to the mirror and spray air every 5 sec
- 7. If decay is between teeth: matrix bands and wedges
 - a. Bands and wedges are used when decay is between teeth and to shape the tooth, but not for occlusal surfaces (chewing surfaces)
 - b. If needed, hand matrix band(s) to doctor, then tweezers, then hold the ends of wedges at the correct angle ()
 - c. 1 band per tooth with decay; 1 wedge for each interproximal space (space btwn teeth)
 - d. AC – garrison (for resins with decay between adult teeth)
 - i. Get garrison clamp and wedges from garrison box
 - ii. String garrison matrix with floss in case it falls
- 8. Restorative

White filling

- Riva can only be used on baby teeth; Equia and other types can be used on adult teeth
- The area does not need to be completely dry nor free of saliva for glass ionomer (i.e., Riva, Equia), unlike other types of resin such as flowable.
- If doing resin and sealant on the same area, etch first then cavity conditioner right after

Glass Ionomer (Riva and Equia Forte)

- 1) Pass cavity conditioner (AA – and/or etch) and rinse/suction
 - a) Dip the orange micro-brush in the cavity conditioner (blue) and hand them the brush
 - b) Then hold the well out for them to dip again (*but make sure to cover the Equia forte coat (yellow) if it is already in the well*)
 - c) Rinse off cavity conditioner and suction
- 2) Mix Riva or Equia for 10 sec (when they say to mix) and test it before handing to doctor
 - a) Tap 2x to activate it and push the bottom against the counter so it goes in the capsule, put in the machine with tip facing backwards
 - b) Flip switch on the left side of the machine and press green button on the bottom (red button if you need to stop it right away)
 - c) Push into applicator, squeeze hard ~2x with tip facing napkin until you hear a hard click sound, squeeze a little bit out (*for Equia no more than a mm is necessary*)
 - i) It's important to squeeze some out b/c the first bit of Riva and Equia that comes out may not be mixed and to test the hardness of the Equia later
 - ii) Hand applicator to doctor (grab the neck so that the handle faces the doctor)
 - d) Throughout the rest of the procedure, keep touching the Equia that you have squeezed out on the napkin to feel how hard it is. Let the doctor know once it feels hard and not soft or squishy.
- 3) Grab wet Q-tip (for after they put material in) and hollenback
- 4) Pass Wet Q-tip, then pass Hollenback (or alternatively, the plastic)
- 5) *For Equia forte only: Equia forte coat*
 - a) *Dip orange micro-brush in Equia forte coat and hold the well out in case they need more (note: Equia forte coat is light sensitive so keep it covered)*
- 6) Have curing light ready in hand
 - a) AC & AB – may need to hold gauze/2x2 out and wipe off excess debris from the Hollenback (pinch the end and wipe off)
- 7) Cure at least 2x (10 sec) with bands on
- 8) If using matrix bands and wedges, hand tweezers back to doctor and hold hand open to take bands and wedges and put back on tray
- 9) Cure again 1x (5 sec) on all sides: top, bottom, and other side) after the doc takes out bands and wedges
- 10) If using rubber dam, pass the clamp forceps to take out the clamp and rubber dam (can also suction the back of the mouth if saliva is pooling).

Resin Composite (Flowable and Packable)

1. Pass etch
2. Dry and rinse
3. Pass bond

- a. Dip bond brush into bond and cover well with bottle (bond is light sensitive)
 - b. Hold well out for the doctor if they are working on multiple areas
 - c. Wait for them to dry the area with air
4. Cure 1x (5 sec)
5. Pass flowable or packable gun
6. Cure 1x (5 sec) (or more depending on situation)
7. Pass micro-brush and hollenback
8. If using matrix bands and wedges, hand tweezers back to doctor and hold hand open to take bands and wedges and put back on tray
9. Pass micro-brush and hollenback
10. After the doctor takes off matrix bands and wedges, cure all sides 1x (5 sec): top, bottom, and other side)
11. Cure 2x (10 sec)

Pulp and stainless-steel crown

Amalgam (silver filling)

9. Polishing (after fillings and sealants)
 - a. Suction with HVE during polishing (can also occasionally use air-water spray to rinse off debris)
 - b. Pass floss (if filling was on distal or mesial surfaces, i.e., decay was btwn teeth) or sandpaper polishing strip
 - c. AC – when decay is btwn teeth, may need:
 - i. (sandpaper) polishing strip
 - ii. “#12” = Blade, scalpel handle, needle holder
 - d. Have floss ready if decay and filling was between teeth (on mesial or distal surfaces)
10. Rinse and suction with saliva ejector to get rid of bad taste, debris, saliva
11. Bite/articulating paper (for white fillings on molars)
 - a. Have bite paper ready to hand over if filling a molar, to check/adjust bite (i.e., if it's too high where the filling was placed)
12. After 5 min, turn off nitrous and oxygen (if used)
 - a. Ask the patient how they feel, if they feel okay, feel dizzy, can stand up, etc.
 - b. If they feel okay, flip the switch that is connected to the gas down
 - c. Turn the knobs on the machine off (to the right)
 - d. Close the gauge on the tanks (turn to the left)
13. Make sure the ledger is completed (Select procedures that were done > “Set Complete”)
14. Give post-op instructions
15. Walk the patient out
 - a. Put the folder in an “Out” tray for:
 - i. “Cleaning” (if the entire treatment plan was completed)
 - ii. “Treatment” (the patient needs to make another appointment to finish the treatment plan)
 - b. Notify front desk if the next appt will be cleaning or treatment

What to do if a patient throws up?

- Help doctor turn patient on their side towards you and hold the trash can near the chair for them to throw up in
- After the treatment is done clean out the trap if you suctioned vomit
- Suction vacuum cleaner (yellow, in the cabinet under the BioSonic) then water
- Take out the trash

Passing Instruments

Take instruments with pinkie finger

Flip to pass after grabbing instruments

Hold instruments with thumb

Hold instruments far away from patients' face (and hands/arms), by the chest instead

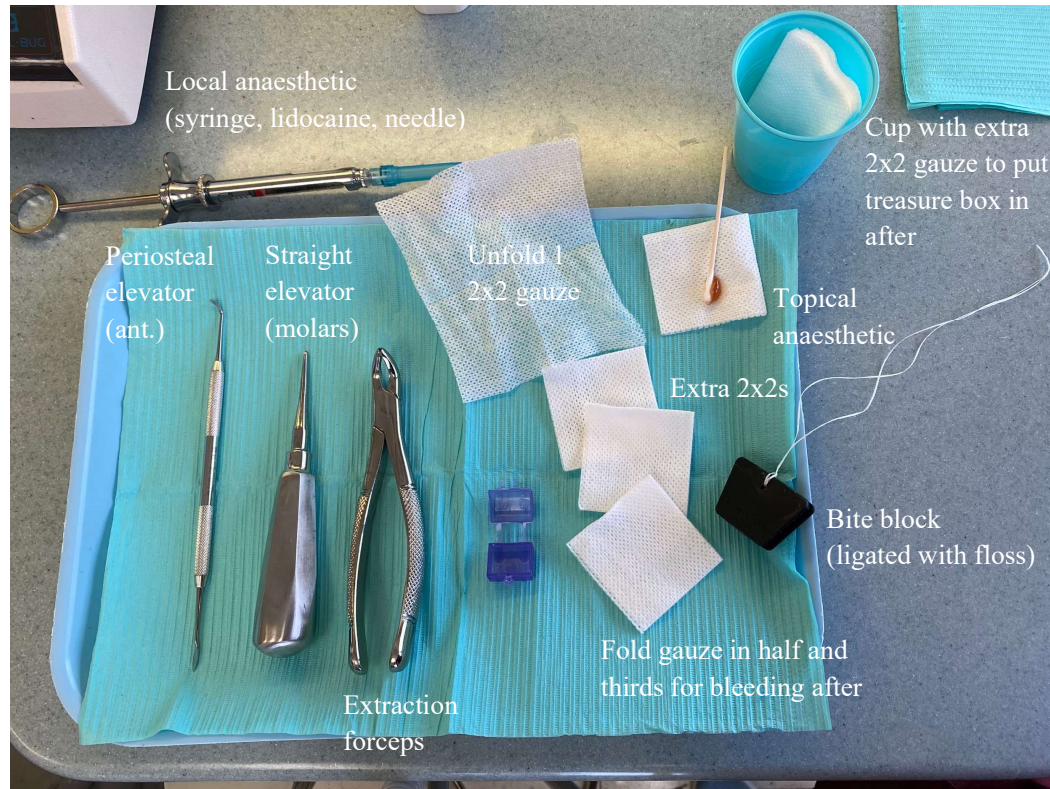
Riva 1) Numb, rinse LA 2) Bite block, rubber dam 3) Prep tooth 4) Bands/wedges/tweezers? 5) Cavity conditioner, rinse (AA – and/or etch, rinse) 6) Mix Riva 7) Wet Qtip, hollenback 8) Cure 2x, take off bands/wedges, cure all sides 1x 9) Polish/suction 10) Floss 11) Bite paper	Sealant 1) Isolate area or retract cheek 2) Etch, wait 20s 3) Dry, bond 4) (Dry) cure (5s) 5) Sealant 6) (Dry) cure (10s) 7) Polish/suction 8) Bite paper
Equia 1) Numb, rinse LA 2) Bite block, rubber dam 3) Prep tooth 4) Bands/wedges/tweezers? 5) Cavity conditioner, rinse (AA – and/or etch, rinse) 6) Mix Equia Forte fil 7) Wet Qtip, hollenback 8) Equia Forte resin coat 9) Cure 2x, take off bands/wedges, cure all sides 1x 10) Polish/suction 11) Floss 12) Bite paper	Extraction 1) Numbing, rinse LA 2) Bite block 3) Extraction 4) Wipe/clean tooth, place in treasure box 5) Change gauze 6) Give cup/tooth/gauze, post-op instructions (numbing, pain, soft foods, avoid hot/spicy)
Flowable/packable 1) Numb, rinse LA 2) Bite block, rubber dam 3) Prep tooth 4) Bands/wedges/tweezers? 5) Etch 6) Dry, rinse 7) Bond, cure 1x 8) Flowable (or packable), cure 1x 9) Microbrush, hollenback 10) Take off bands/wedges, cure all sides 1x 11) Cure 2x 12) Polish/suction 13) Floss 14) Bite paper	Pulp and stainless steel crown 1) Numb, rinse LA 2) Bite block, rubber dam 3) Prep tooth 4) 2nd: switch handpieces => mix IRM 5) ViscoStat 6) Wet Q-tip, hollenback and gauze 7) Clean w/ air/water syringe 8) 2nd: Crown 9) Crown crimper, howe pliers, bite stick 10) Clean w/ air/water syringe 11) Clean cap 12) 2nd: Mix cement and put in crown 13) Bite stick, 2x2 14) Suction excess cement 15) Explorer, floss 16) Suction, clean

Amalgam	Post-op Instructions <u>Numbing</u> : Since we numbed the area, be careful you don't accidentally bite yourself and don't touch or play with your lip. The numbing should wear off after 2 hours. So, I wouldn't eat for the next 2 hours, until the numbing wears off. You can take pain medication if it hurts, but the pain should resolve in the next couple of days.
<u>White fillings</u> : For the next 24 hours, don't eat heavily colored food or drinks like fruit punch, curry, or candy that might stain the filling. <u>Silver fillings</u> : Don't eat for the next 15 min (if no numbing used). <u>Extractions</u> : Bite down on the gauze until it stops bleeding. Change every 30 min. Avoid rinsing or spitting too hard, using straws or sucking, and playing with the area b/c it can loosen the blood clot or stop it from forming. Avoid heavy exercise to limit the amount of blood flowing to the area. Avoid spicy, hot food and eat softer foods to manage the pain. The pain should go away in a couple days.	<u>Sealants</u> : You can eat and drink right away. If after the next couple of days, your bite still feels weird, you can call the office and we can adjust it. <u>Crowns</u> : Do not pull on the caps and avoid sticky foods like gum, caramel, etc. <u>SDF</u> : No eating or drinking for the next 30 minutes.

Extraction

- Context: (at patient/parent request, when teeth don't fall out within normal timeframe, or causing pain?)

Set up



- Mirror and explorer
- Bite block
- Elevator (not necessary if super loose)
 - Periosteal elevator (for anterior)
 - Straight elevator (for molars)
- Forceps
 - 1 = anterior (front) teeth
 - 150 = maxillary (top) molars
 - 150s = top baby teeth
 - 151 = mandibular (bottom) molars
 - 151s = bottom baby teeth
- Syringe, lidocaine, needle
- If using nitrous oxide: nose, barrier for tanks
- Topical anaesthetic on Q-tip and 2x2 gauze (AA, AC)
- 2x2 gauze
 - Completely unfold one piece of gauze for catching the tooth in case it falls (AA – fold corner down)
 - Fold 2 pieces into a roll for each tooth pulled (e.g., fold gauze in half and roll/fold into 3rds) for applying pressure to stop the bleeding
 - Have some extra
- Cup with extra gauze and treasure box

Procedure

1. Numbing
 - a. Topical anaesthetic
 - b. Nitrous
 - c. Local anaesthetic
2. Bite block
3. Tooth extraction with forceps (make sure to have the correct ones)
4. Clean/wipe blood off the tooth and place it in a treasure box
5. If nitrous was used, once they turn nitrous off and oxygen on, wait for 5 min before asking if they feel okay. Turn the gas and oxygen tanks off immediately afterwards.
6. Change their gauze before they leave
7. Give them a cup with extra gauze and their tooth in a treasure box

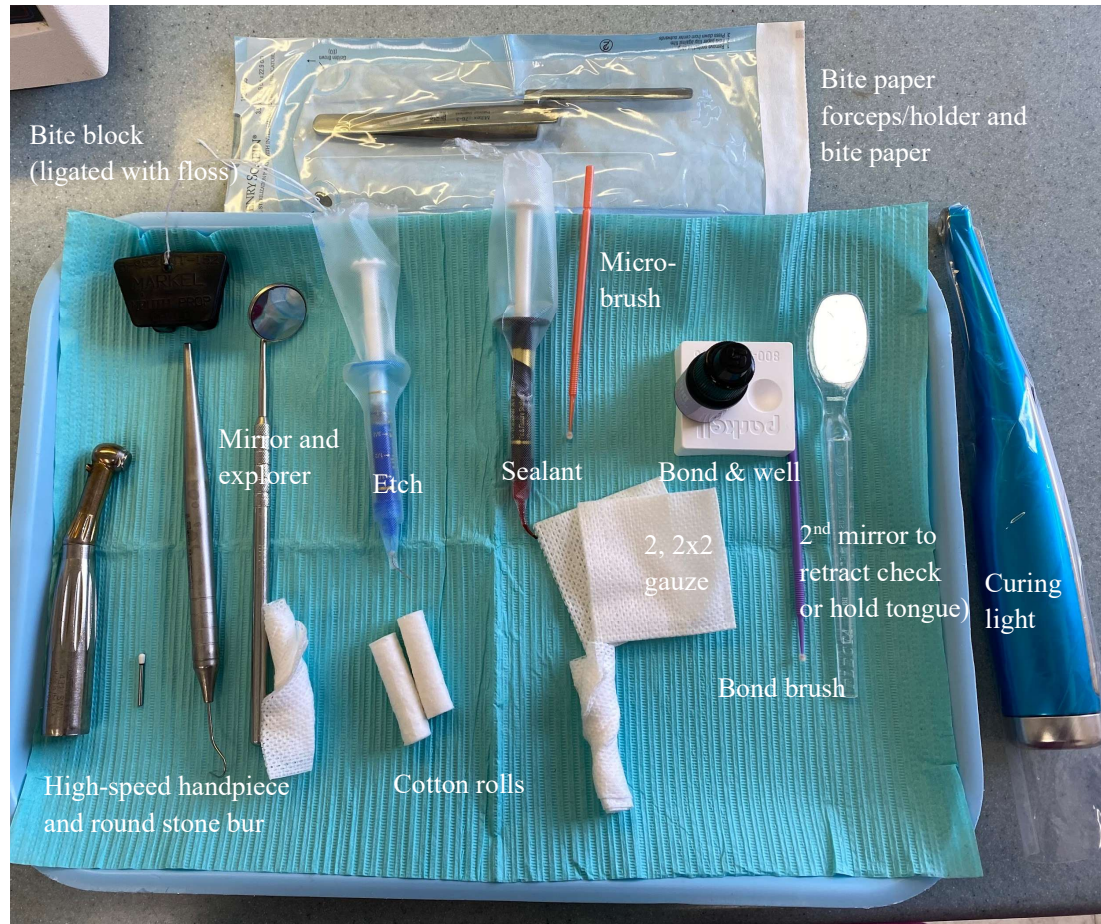
Post-op Instructions

- Bite down on the gauze until it stops bleeding. Change every 30 min.
- Avoid rinsing or spitting too hard, using straws or sucking, and playing with the area b/c it can loosen the blood clot or stop it from forming.
- Avoid heavy exercise to limit the amount of blood flow to the area and stop bleeding.
- Avoid spicy, hot food and eat softer foods to manage the pain. The pain should go away in a couple days.
- If used numbing:
 - Since we numbed the area, be careful you don't accidentally bite yourself and don't touch or play with your lip.
 - The numbing should wear off after 2 hours. So, I wouldn't eat for the next 2 hours, until the numbing wears off.
 - You can take pain medication if it hurts, but the pain should resolve in the next couple of days.

Sealant

- A sealant is a thin, tough, plastic coating painted on the pits and grooves of a tooth to prevent tooth decay. It can help prevent small decay from growing larger.

Set-up



- Mirror and explorer + extra mirror
- Bite block
- Have bite paper holder on the side
- High-speed handpiece
- Burs: stone (round)
- Etch
- Sealant
- Bond, well, bond brush (keep the bond covered b/c it's light sensitive)
- Micro-brush
- 2x2 gauze and cotton rolls (wrap 1 cotton roll in gauze for each side you're working on + have 2 extra 2x2 gauze)
- Curing light with barrier

Procedure

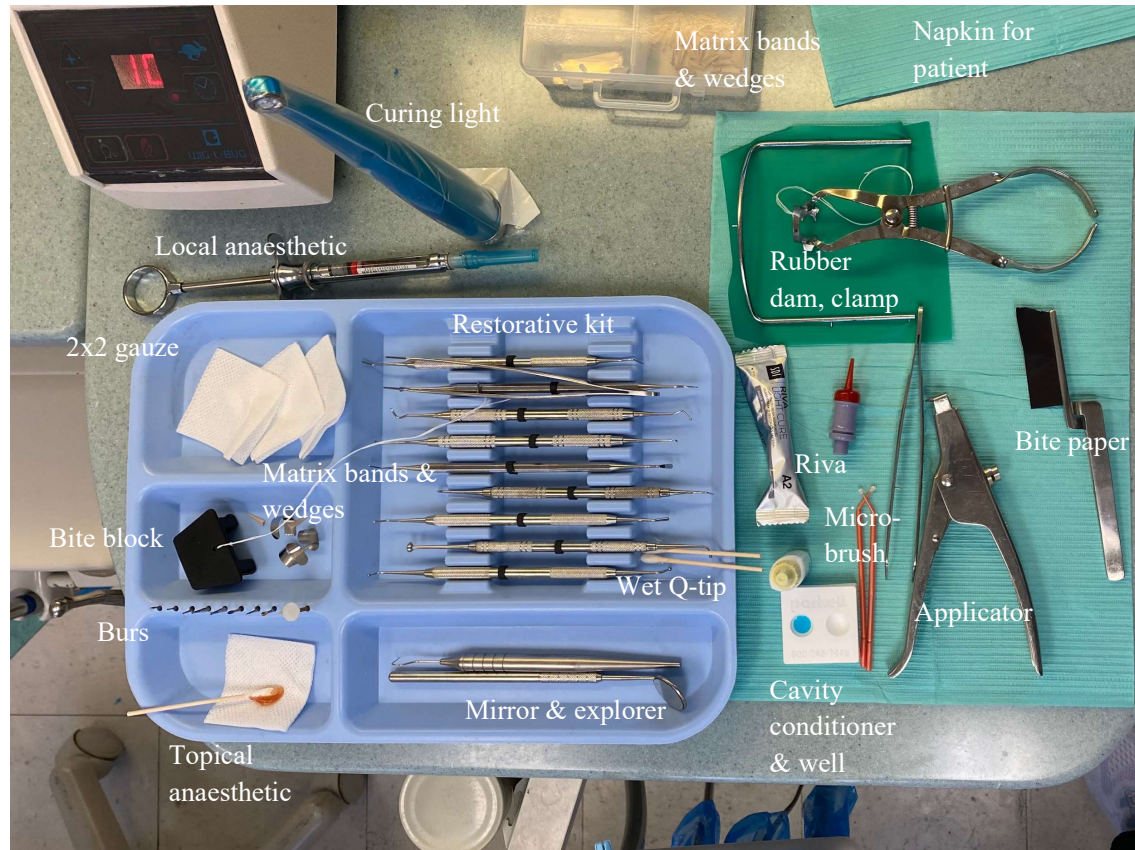
1. Doctor puts in bite block
2. Isolate the area
 - a. AC — will isolate the area with 2x2s and cotton rolls
 - i. Maxillary – 2 (1 on each side)
 - ii. Mandibular – 4 (2 on each side, 1 rolled in gauze)
 - b. AB & AA — using a mirror, either retract the cheek or hold the mirror against the tongue, depending on which side they're working on
3. Suction using the HVE to dry the area
 - a. Take out HVE while the doctor uses the air/water syringe to dry
4. Pass etch and wait for 20 sec after applying, get HVE ready
5. Suction off the etch using the HVE and while the doctor rinses
 - a. AC — suction the area with the etch, then suction the cotton rolls, each time you dry
6. Dip purple brush into the bond and pass the bond brush to the doctor (cover tip from light), while holding HVE
7. Once they apply bond, dry the area by suctioning with HVE (have light in other hand)
8. Cure with the light (1x or 5 sec)
9. Pass the sealant to the doctor
10. Help dry the area by suctioning with HVE (have light ready)
11. Cure with the light (2x or 10 sec)
12. If the doctor picks up a handpiece to start polishing the area, have the HVE ready to suction nearby (be careful not to block the area they're working on)
13. Rinse and suction

Post-op Instructions

- We didn't use any numbing so you can eat or drink right away
- Your bite might feel a little weird for the next couple of days. If it still feels different after that, you can give us a call and we can help fix your bite.

Glass ionomer (Riva)

Set up



- Restorative kit
- Mirror and explorer
- Bite block
- Rubber dam frame, clamp forceps, clamp OR extra mirror (AC: and cotton rolls) OR mouth prop
- Syringe, lidocaine, needle
- Applicator
- Bite paper holder
- High and slow-speed handpieces
- Burs: 34SS, 557, 4SS, (AC - 330) FB gold, gold, stone, polish
- Well
- AA: Etch
- Cavity conditioner + micro-brush
- Wet Q-tip
- 2x2 gauze (~3-4 pieces)
- Topical anaesthetic on Q-tip and 2x2 gauze (AA, AC)
- Matrix bands and wedges (if decay is between teeth)
- Riva (plus 1 extra) –
 - Shades:
 - A1 (anterior)
 - A2 (molars)
- Curing light with barrier
- Extra piece of floss
- If using nitrous oxide: nose, barrier for tanks

Procedure

- 1) Numbing
 - a. Topical anaesthetic
 - b. Nitrous
 - c. Local anaesthetic
- 2) Bite block
- 3) Rubber dam and clamp
- 4) Prep the tooth
- 5) Hand matrix band(s) to doctor, then hand wedges (if necessary)
- 6) Pass cavity conditioner (AA – and/or etch) and rinse/suction
 - a. Dip the orange micro-brush in the cavity conditioner (blue) and hand them the brush
 - b. Then hold the well out for them to dip again
 - c. Rinse off cavity conditioner and suction
- 7) Mix Riva for 10 sec (when they say to mix) and test it before handing to doctor
 - a. Tap 2x to activate Riva and push the bottom against the counter so it goes in the capsule, put in the machine with tip facing backwards
 - b. Flip switch on the left side of the machine and press green button on the bottom (red button if you need to stop it right away)
 - c. Push into applicator, squeeze hard ~2x with tip facing napkin until you hear a hard click sound, squeeze a little bit out
 - d. Hand applicator to doctor (grab the neck so that the handle faces the doctor)
- 8) Grab wet Q-tip (for after they put material in) and hollenback
- 9) Pass Wet Q-tip, then pass Hollenback (or alternatively, the plastic)
- 10) Have curing light ready in hand
 - a. AC & AB – may need to hold gauze/2x2 out and wipe off excess debris from the Hollenback (pinch the end and wipe off)
- 11) Cure at least 2x (10 sec) with bands on
- 12) If using matrix bands and wedges, hand tweezers back to doctor and hold hand open to take bands and wedges and put back on tray
- 13) Cure again 1x (5 sec) on all sides: top, bottom, and other side) after the doc takes out bands and wedges
- 14) If using rubber dam, pass the clamp forceps to take out the clamp and rubber dam (can also suction the back of the mouth if saliva is pooling).
- 15) Polishing
 - a. Suction with HVE during polishing (can also occasionally use air-water spray to rinse off debris)
 - b. Pass floss (if filling was on distal or mesial surfaces, i.e., decay was btwn teeth) or sandpaper polishing strip (AC – #12)
- 16) Pass bite paper to check bite
- 17) Optional: Rinse and suction with saliva ejector

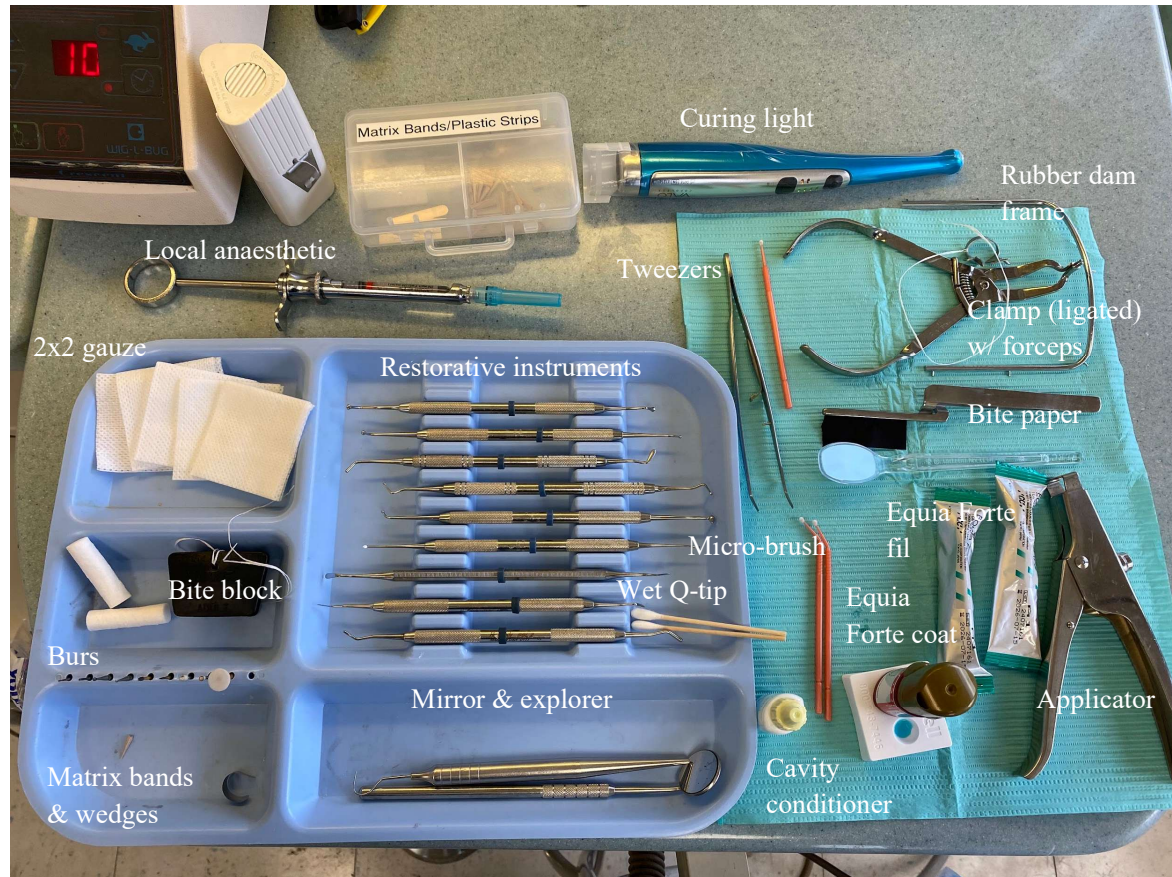
Post-op Instructions

- For the next 24 hours, don't eat heavily colored food or drinks like fruit punch, curry, or candy that might stain the filling.
- If your bite feels weird after the next couple of days, you can give us a call and we can fix it.

- If used numbing:
 - Since we numbed the area, be careful you don't accidentally bite yourself and don't touch or play with your lip.
 - The numbing should wear off after 2 hours. So, I wouldn't eat for the next 2 hours, until the numbing wears off.
 - You can take pain medication if it hurts, but the pain should resolve in the next couple of days.

Glass ionomer (Equia Forte)

Set up



- Restorative kit
- Mirror and explorer
- Bite block
- Rubber dam frame, clamp forceps, clamp OR extra mirror (AC: and cotton rolls) OR mouth prop
- Syringe, lidocaine, needle
- Applicator
- Bite paper holder
- High and slow-speed handpieces
- Burs: 34SS, 557, 4SS, (AC - 330) FB gold, gold, stone, polish
- Well
- Cavity conditioner + micro-brush
- AA: Etch
- 2x2 gauze (~3-4 pieces)
- Wet Q-tip + extra cue tip
- Topical anaesthetic on Q-tip and 2x2 gauze (AA, AC)
- Matrix bands and wedges (if decay is between teeth)
- Equia Forte fil + Equia Forte coat
 - Shades:
 - A1: anterior
 - A2: molars
- Curing light with barrier
- Extra piece of floss
- If using nitrous oxide: nose, barrier for tanks

Procedure

1. Numbing
 - a. Topical anaesthetic
 - b. Nitrous
 - c. Local anaesthetic
2. Bite block
3. Rubber dam and clamp
4. Prep the tooth
5. Hand matrix band(s) to doctor, then hand wedges (if necessary)
6. Pass cavity conditioner (AA – and/or etch) and rinse/suction
 - a. Dip the orange micro-brush in the cavity conditioner (blue) and hand them the brush
 - b. Then hold the well out for them to dip again (*but make sure to cover the Equia forte coat (yellow)*)
 - c. Rinse off cavity conditioner and suction
7. Mix Equia for 10 sec (when they say to mix) and test it before handing to doctor
 - a. Tap 2x to activate it and push the bottom against the counter so it goes in the capsule, put in the machine with tip facing backwards
 - b. Flip switch on the left side of the machine and press green button on the bottom (red button if you need to stop it right away)
 - c. Push into applicator, squeeze hard ~2x with tip facing napkin until you hear a hard click sound, squeeze a little bit out (*for Equia no more than a mm is necessary*)
 - d. Hand applicator to doctor (grab the neck so that the handle faces the doctor)
 - e. Throughout the rest of the procedure, keep touching the Equia that you have squeezed out on the napkin to feel how hard it is. Let the doctor know once it feels hard and not soft or squishy.
8. Grab wet Q-tip (for after they put material in) and Hollenback
9. Pass Wet Q-tip, then pass Hollenback (or alternatively, the plastic)
10. Equia forte coat
 - a. Dip orange micro-brush in Equia forte coat and hold the well out in case they need more (note: Equia forte coat is light sensitive so keep it covered)
11. Have curing light ready in hand
 - a. AC & AB – may need to hold gauze/2x2 out and wipe off excess debris from the Hollenback (pinch the end and wipe off)
12. Cure at least 2x (10 sec) with bands on
13. If using matrix bands and wedges, hand tweezers back to doctor and hold hand open to take bands and wedges and put back on tray
14. Cure again 1x (5 sec) on all sides: top, bottom, and other side) after the doc takes out bands and wedges
15. If using rubber dam, pass the clamp forceps to take out the clamp and rubber dam (can also suction the back of the mouth if saliva is pooling).
16. Polishing
 - a. Suction with HVE during polishing (can also occasionally use air-water spray to rinse off debris)
 - b. Pass floss (if filling was on distal or mesial surfaces, i.e., decay was btwn teeth) or sandpaper polishing strip (AC – #12)
17. Pass bite paper to check bite

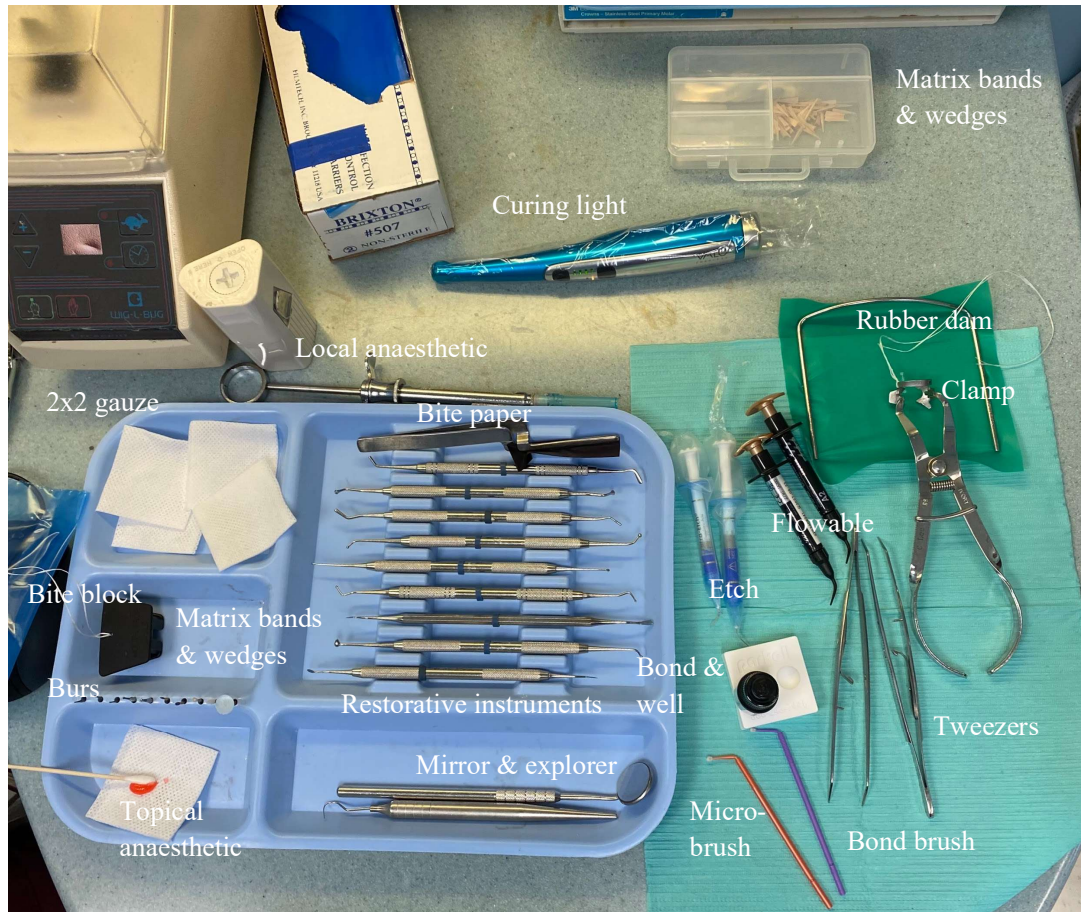
18. Optional: Rinse and suction with saliva ejector

Post-op Instructions

- For the next 24 hours, don't eat heavily colored food or drinks like fruit punch, curry, or candy that might stain the filling.
- If your bite feels weird after the next couple of days, you can give us a call and we can fix it.
- If used numbing:
 - Since we numbed the area, be careful you don't accidentally bite yourself and don't touch or play with your lip.
 - The numbing should wear off after 2 hours. So, I wouldn't eat for the next 2 hours, until the numbing wears off.
 - You can take pain medication if it hurts, but the pain should resolve in the next couple of days.

Resin composite (Flowable & Packable)

Set up



- Mirror/explorer
- Restorative kit
- Bite block
- Rubber dam frame, clamp forceps, clamp OR extra mirror (AC: and cotton rolls) OR mouth prop
- Syringe, lidocaine, needle
- Bite paper holder
- High and slow-speed handpieces
- Burs: 34SS, 557, 4SS (AC - 330), FB gold, gold, stone, polish
- Well
- Etch
- Bond and bond brush
- Micro-brush
- Matrix bands and wedges (if decay is between teeth)
- 2x2 gauze (~3-4 pieces)
- Topical anaesthetic on Q-tip and 2x2 gauze (AA, AC)
- Extra floss
- Curing light
- *Packable* capsule and gun:
 
- OR *Flowable* (if there's not much left in the tube, have extra)

Shades of Flowable:

- A1: front adult teeth
- A2: back adult teeth
- B1: baby teeth

Procedure

12. Numbing
 - a. Topical anaesthetic
 - b. Nitrous
 - c. Local anaesthetic
13. Bite block
14. Rubber dam
15. Prep the tooth
16. Pass etch
17. Dry and rinse
18. Pass bond
 - a. Dip bond brush into bond and cover well with bottle (bond is light sensitive)
 - b. Hold well out for the doctor if they are working on multiple areas
 - c. Wait for them to dry the area with air
19. Cure 1x (5 sec)
20. Pass flowable or packable gun
21. Cure 1x (5 sec) (or more depending on situation)
22. Pass micro-brush and hollenback
23. If using matrix bands and wedges, hand tweezers back to doctor and hold hand open to take bands and wedges and put back on tray
24. Pass micro-brush and hollenback
25. After the doctor takes off matrix bands and wedges, cure all sides 1x (5 sec): top, bottom, and other side)
26. Cure 2x (10 sec)
27. If using rubber dam, pass the clamp forceps to take out the clamp and rubber dam (can also suction the back of the mouth if saliva is pooling)
28. Polishing
 - a. Suction with HVE during polishing (can also occasionally use air-water spray to rinse off debris)
 - b. Pass floss (if filling was on distal or mesial surfaces, i.e., decay was btwn teeth) or sandpaper polishing strip (AC – #12)
29. Pass bite paper to check bite
30. Optional: Rinse and suction with saliva ejector

Post-op Instructions

- For the next 24 hours, don't eat heavily colored food or drinks like fruit punch, curry, or candy that might stain the filling.
- If your bite feels weird after the next couple of days, you can give us a call and we can fix it.
- If used numbing:
 - Since we numbed the area, be careful you don't accidentally bite yourself and don't touch or play with your lip.
 - The numbing should wear off after 2 hours. So, I wouldn't eat for the next 2 hours, until the numbing wears off.
 - You can take pain medication if it hurts, but the pain should resolve in the next couple of days.

Context

- Pulpotomy (uses IRM and cement) vs. SSC (uses only cement)

Stainless steel crown

- Stainless Steel Crown – silver (in cabinet above the AED)
- Bioflex (Nusmile) – white (plastic) crown (located to the left of the other crowns)
 - Not covered by most insurance since it's not a necessity
 - Can't use crimper on plastic, which makes white crowns are harder to size

Procedure

1. Numbing
 1. Topical anaesthetic
 2. Nitrous
 3. Local anaesthetic
2. Bite block and rubber dam
3. Doctor checks if nerve is still healthy, to determine if pulp or Metapix is needed
 1. Metapix – if they have an infection or abscess
 1. File to remove nerve, rinse and use paper points to dry the root canal
4. Prep the tooth – suction to clean and dry
5. 2nd assistant: When doc is opening pulp (i.e. switching from high to slow speed) or you can ask if you should start mixing, start mixing IRM
 1. Wipe IRM off the spatula before it hardens
 2. High speed => remove hard surface from tooth; slow speed => touch-ups
6. Pass ViscoStat (light brown/orange tube) to stop bleeding and fill pulp chamber
 1. If IRM is soft, also have a wet Q-tip ready
 2. Have 2x2 (ideally) or HVE (high speed) suction to remove excess
 1. (If you suction the IRM, check the trap later to prevent clogging)
 3. OR if ____ : Mineral Trioxide Aggregate (MAB)
 1. Context???: (similar to IRM) for infections?
 2. MAB + Sodium hypo-chlorox (aka clorox) + cotton balls
7. Clean IRM off the tooth
8. 2nd assistant: Get the crown when doctor asks or if IRM is done
 1. Crowns: D = 1st molars ; E = 2nd molars (also, on the computer/Dentrix, the crowns are labeled from the patient's point of view. In the box, it's the opposite.)
 2. Have one tweezers just for the box (place on top of the box) to avoid cross-contamination
9. Pass the crown crimper (don't confuse with the wire cutter which has a hole)
10. Trying on caps
 1. If we aren't using a rubber dam, have the high-speed suction on all the time to suction up the crown in case it falls so the patient doesn't swallow it
11. Pass either Howe pliers, spoon or carver *and* bite stick (to fit/shape & push cap on tooth)
 1. Have the cap that fits on the side
12. Clean tooth and dry (also spray the cap with air or water to clean)
13. 2nd assistant: Once the doctor says they're ready, mix the cement (fuji) and put it inside the crown
 1. Start over if it's too thick. If it's too soft, add more powder.

2. OR: Meron Plus – squeeze out a little on the side or replace tip after; can be used anytime instead of cement (e.g., if by yourself and no one can mix, or you need it immediately)
14. 2nd assistant: Place crown with the cement facing down, on the 1st assistant's glove
15. Hold out the bite stick and gauze/2x2
16. Suction to clean off excess cement with the HVE
17. Make sure to have extra floss ready for the doctor
18. After the doctor takes the rubber dam off, suction and clean up

Post-op Instructions

Do not pull on the caps and avoid sticky foods like gum, caramel, etc.

If used numbing:

- Since we numbed the area, be careful you don't accidentally bite yourself and don't touch or play with your lip.
- The numbing should wear off after 2 hours. So, I wouldn't eat for the next 2 hours, until the numbing wears off.
- You can take pain medication if it hurts, but the pain should resolve in the next couple of days.

Silver Diamine Fluoride (SDF)

Note: Be careful with SDF because it stains clothing and skin (if you get SDF on your gloves, change them right away)

Pre-op instructions (for parents deciding on using SDF):

- Silver diamine fluoride is an antibacterial medicine designed to kill bacteria.
- Explain purpose of using SDF on patient (e.g., We can use it to stop ___'s cavity from growing.)
- Disclaimers:
 - It's not 100% effective but it can help cavities stop growing.
 - There will be black discoloration on the tooth, from the silver.
 - *Ask to make sure they are not allergic to silver (Is ___ allergic to silver?)
 - If we do it, you cannot eat or drink for the next 30 min.
- Ask parents if they want to do silver diamine fluoride.

Set-up:

- Micro-brush
- Non-absorbent paper
- ½ or 1 drop of SDF (depending on how many teeth are being treated)

Procedure: Pass brush and hold SDF (not in the way of patient's arms if they are younger)

Post-op instructions: No eating or drinking for the next 30 minutes.

Alginate Impressions

Bring materials to Dr. Mary:



Impression trays

measuring cup, material, spatula, bowl

???

- Metal bands?
- Impression tray
- Alginate impression material
- Alginate spatula
- Water (follow directions on container: 1 scoop dustless alginate impression material = 1 scoop of water)

Steps

- Mix jar of alginate impression material (gently turn upside down and back up to mix)
- Scoop out alginate impression material and pour appropriate amount of tap water into measuring tube
 - 1 scoop dustless alginate impression material = 1 equivalent scoop of water
- Bring materials (impression tray, impression material, alginate spatula, water) to doctor
- Doctor mixes water and impression material using alginate spatula
 - Doctor measures mouth of patient
 - If it's the right size, she will spread the mixture across the impression tray
 - Doctor presses into teeth and hold until it sets (has some on hand to test/feel if it's ready)
- Spray impression with cavicide
- Cover impression tray with material in damp napkins

Clean up

- Wipe impression material off spatula and bowl
- Rinse and wipe with alcohol